

Gotu kola, kava kava, St. John's wort, valerian may increase CNS depression. **FOOD:** None known. **LAB VALUES:** May decrease total free thyroxine (T_4) serum levels. May increase serum cholesterol, triglycerides, ALT, AST, WBC, GGT. May produce false-positive pregnancy test result.

AVAILABILITY (Rx)

Tablets: 25 mg, 50 mg, 100 mg, 200 mg, 300 mg, 400 mg.

 **Tablets, Extended-Release:** 50 mg, 150 mg, 200 mg, 300 mg, 400 mg.

ADMINISTRATION/HANDLING

PO

- Give immediate-release tablets without regard to food.
- Do not break, crush, dissolve, or divide extended-release tablets.
- Extended-release tablets should be given without regard to food or with a light meal in evening.

INDICATIONS/ROUTES/DOSAGE

Note: • When restarting pts who have been off QUetiapine for less than 1 wk, titration is not required and maintenance dose can be reinstituted. • When restarting pts who have been off QUetiapine for longer than 1 wk, follow initial titration schedule. • When discontinuing, gradual tapering recommended to avoid withdrawal symptoms and minimize risk of relapse.

Schizophrenia

PO: (Immediate-Release): ADULTS, ELDERLY: Initially, 25 mg twice daily, then increase in 25–50-mg increments divided 2–3 times/day on the second and third days, up to 300–400 mg/day in 2–3 divided doses by the fourth day. Further adjustments of 25–50 mg twice daily may be made at intervals of 2 days or longer. **Maintenance:** 150–750 mg/day (adults); 50–200 mg/day (elderly). **(Extended-Release):** Initially, 300 mg/day. May increase at intervals as short as 1 day up to 300 mg/day. Range: 400–800 mg/day. **(Immediate-Release): CHILDREN**

13 YRS AND OLDER: Initially, 25 mg twice daily on day 1, 50 mg twice daily on day 2, then increase by 100 mg/day to target dose of 400 mg twice daily on day 5. May further increase to 800 mg/day in increments of 100 mg or less daily. Range: 400–800 mg/day. **Maximum:** 800 mg. Total dose in 3 divided doses. **(Extended-Release):** Initially, 50 mg once daily on day 1, 100 mg on day 2, until 400 mg once daily is reached on day 5. Range: 400–800 mg/day. **Maximum:** 800 mg/day.

Mania in Bipolar Disorder

PO: (Immediate-Release): ADULTS, ELDERLY: Initially, 50 mg twice daily for 1 day. May increase in increments of 100 mg/day to 200 mg twice daily on day 4. May further increase in increments of 200 mg/day to 800 mg/day on day 6. Range: 400–800 mg/day. **(Extended-Release):** Initially, 300 mg on day 1 in the evening; 600 mg on day 2 and adjust between 400–800 mg/day thereafter. **(Immediate-Release): CHILDREN 10 YRS AND OLDER:** 25 mg twice daily on day 1, 50 mg twice daily on day 2, then increase by 100 mg/day until target dose of 400 mg/day reached on day 5. May increase up to 600 mg/day. Range: 400–600 mg/day. **(Extended-Release):** 50 mg on day 1; 100 mg on day 2; further increases of 100 mg/day until 400 mg once daily is reached on day 4; Usual range: 400–600 mg once daily.

Depression in Bipolar Disorder

PO: (Immediate-Release): ADULTS, ELDERLY: Initially, 50 mg/day on day 1, increase to 100 mg/day on day 2, then increase by 100 mg/day up to target dose of 300 mg/day. **(Extended-Release):** Initially, 50 mg on day 1 in the evening, 100 mg on day 2, 200 mg on day 3, 300 mg on day 4 and thereafter.

Adjunctive Therapy in MDD

PO: ADULTS, ELDERLY: (Extended-Release): Initially, 50 mg on days 1 and 2; then 150 mg on days 3 and 4; then 150–300 mg/day thereafter.